

Patient Profile

Name: Mr. Saubhik Bhaumik

Age:

Gender:

Summary of Results

The patient's laboratory profile has been evaluated by a predictive model that classifies the overall pattern as **Normal**. Individual test values largely fall within standard reference ranges, with the exception of an elevated globulin value that may warrant further clinical correlation.

Detailed Test Explanations

Test	Result	Reference	Clinical Significance
Serum Bilirubin (Total)	0.9 mg/dl	0–12 mg/dl	Indicates normal hepatic bilirubin processing and clearance.
Serum Bilirubin (Direct)	0.2 mg/dl	0–3 mg/dl	Low direct bilirubin suggests no cholestatic injury.
Serum Bilirubin (Indirect)	0.70 mg/dl	0–1 mg/dl	Normal indirect bilirubin; no hemolysis or hepatic conjugation defect.
SGPT (ALT)	36 un	13–40 un	Within normal limits; no significant hepatocellular injury.
SGOT (AST)	32 un	0–37 un	Normal; no evidence of myocardial or hepatic stress.
Serum Alkaline Phosphatase	No result provided	—	Unable to evaluate; consider repeat if clinical suspicion exists.
Serum Protein	72 gidl	64–83 gidl	Normal total protein; adequate nutritional and immune status.
Serum Albumin	47 gidl	35–52 gidl	Normal albumin; adequate synthetic liver function.
Globulin	250 gid	18–36 gid	Markedly elevated globulin; may reflect chronic inflammation, infection, or plasma cell dyscrasia.
AIG Ratio (Albumin/Globulin)	1.88	1–24	Within normal range; despite high globulin, albumin remains adequate.

Overall Interpretation

The majority of liver function tests—bilirubin fractions, transaminases, serum protein, and albumin—are within expected ranges, indicating preserved hepatic synthetic and excretory capacity. The pronounced rise in globulin is an outlier and suggests a potential immunologic or chronic inflammatory process. However, the overall profile remains within the predictive model's normal classification, implying that no acute hepatic pathology is evident.

Recommendations

- **Clinical Correlation:** Discuss the elevated globulin with a healthcare provider; consider targeted history and examination for signs of infection, autoimmune disease, or chronic inflammation.
- **Follow-up Testing:** Reassess serum globulin and repeat the missing alkaline phosphatase value to clarify liver enzyme profile.
- **Lifestyle Modifications:** Maintain a balanced diet rich in fruits, vegetables, and adequate hydration. Limit alcohol intake and avoid hepatotoxic medications or supplements.
- **Monitoring:** Schedule routine liver function panel annually or sooner if symptoms arise (e.g., jaundice, abdominal pain).

Disclaimer

This report is a preliminary interpretation of laboratory findings and should not replace a comprehensive clinical evaluation by a qualified medical professional. Consult your physician for personalized advice and definitive diagnosis.